CASOMB Complaint Submission – HOPE Psychotherapy, Inc.

Brendan Ngwa Nforbi <bnn.cdl@gmail.com> Thu, Feb 26 at 4:37 PM
To: <KIRSTEN.MASON@cdcr.ca.gov>
Cc: CASOMB@CDCR <casomb@cdcr.ca.gov>

Dear Dr. Mason,

Thank you for reviewing my complaint and for the opportunity to provide additional information. I will address each of your questions directly.

Treatment Plan — Creation, Review, and Signature

No. In the approximately five years I have been enrolled in the HOPE Program (since approximately May 2021), no individualized treatment plan has ever been created, presented to me for review, or signed by me and a clinician. The only documentation presented to me was at intake and at subsequent annual reassessments (typically near July of each year), and that documentation consisted primarily of assessment-based questionnaires — not a treatment plan with individualized goals, completion criteria, or a pathway through the program.

At no point has any clinician sat down with me to establish written treatment objectives, define what conduct or progress is required to advance through phases, or identify specific criteria for treatment completion. I have never signed a treatment plan document of any kind.

Treatment Progress Discussion and Review

No meaningful review of my individual treatment progress has occurred. In the summer of 2025, I made multiple attempts to schedule a meeting with Katie Palacios, the director of the HOPE Program in Stockton, specifically to discuss my treatment progress and to understand why — after five years in the program — I remained in the Core Phase at twice-weekly attendance. She did not respond to these requests.

To be clear about my actual participation: I have completed both Block I and Block II of HOPE's curriculum multiple times over. I routinely demonstrate an understanding of the course material through my contributions in group sessions and continue to implement what I have learned outside of class. I have taken full responsibility for my actions, and the remorse I have expressed for the crime I committed has been cited as a mitigating factor in Stockton Parole's own documentation, including the November 2025 Discharge Review Report — the same report that ultimately recommended retaining me on parole based on HOPE's "non-completion" characterization.

Despite all of this — completing the curriculum multiple times, actively participating, demonstrating understanding, and being commended by parole for taking responsibility — I have never been advanced, never been provided with a written explanation of what additional progress is required, and never been given any indication of what it would take to move to the next phase. No such written criteria exist, or if they do, they have never been shared with me.

The absence of any individualized progress review is compounded by extraordinarily high clinician turnover. Over my five years in the program, I have been assigned to approximately nine different clinicians: Pamela Akins, Dulce Espinoza, Ms. B, Ms. Vicky, Ms. Carla, Katie Palacios, Nicole Juszczak, Dr. Beasley, and Ms. Joann. Each reassignment effectively meant starting over — there was no continuity of care, no transfer of treatment progress from one clinician to the next, and no mechanism to ensure that work completed under a prior clinician was recognized or preserved.

Homework and Documentation

The issue of homework completion is directly relevant to your questions. Homework completion is routinely cited during my Containment Team Meetings as a measure of my progress, and Stockton Parole has used my alleged failure to complete homework as a basis for denying my discharge from parole. However, Katie Palacios has stated during HOPE

group sessions — on multiple occasions, in front of other participants — that she routinely loses client homework. This is not unique to my experience; it is one of the most common complaints raised by participants during group sessions. There is no cataloging system, database, or tracking mechanism in place to ensure that completed homework is preserved and can be referenced by either clinicians or clients.

This problem extends beyond the Stockton location. A participant named Marcellus transferred from the HOPE Program in Hayward to the Stockton location and was told that none of his homework had transferred with him and that he would need to start over from the beginning.

The combined effect is that HOPE cites homework completion as a factor in treatment progress while simultaneously operating a system in which completed homework is routinely lost, not tracked, and not transferable — and then uses the resulting gaps in its own records as justification for characterizing participants as not having completed treatment.

Please let me know if you need any additional information.

Respectfully,

Brendan Ngehsi Newanforbi
(209) 801-4443
bnn.cdl@gmail.com

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